

MP-106: Total Knee Arthroplasty (TKA)

Bellcourt Medical Policy | Version 1 | Status: RETIRED (superseded 2025-12-31) | Effective for dates of service 2024-01-01 through 2025-12-31

Field	Value
Policy number	MP-106
Service code(s)	BHA-SURG-2744
Applies to	All clients administered by Bellcourt unless the client's plan document or delegation addendum states otherwise
Owner	Clinical Policy Committee
Version	v1

1. Purpose and scope

This policy sets out the clinical criteria Bellcourt reviewers use to decide whether the service described in this policy (Total Knee Arthroplasty (TKA)) is medically necessary. It is applied to prior authorization requests where the date of service falls within the effective period stated above. A client's plan document may exclude this service or impose additional requirements; plan provisions are checked first.

2. Definitions

Medically necessary: services that are appropriate and consistent with the diagnosis, in accordance with generally accepted standards of medical practice, and not primarily for the convenience of the member or provider.

3. Coverage criteria

The service is considered medically necessary when the criteria below are met, read together using the AND / OR connectors shown:

- 1 Radiographic osteoarthritis of Kellgren-Lawrence grade 3 or 4; AND
- 2 Functional limitation interfering with activities of daily living; AND
- 3 Failure of at least 3 months of conservative management; AND
- 4 BMI below 40.

4. Not medically necessary / limitations

- BMI of 40 or greater.
- Kellgren-Lawrence grade 0 to 2.

5. Documentation requirements

Standing radiographs with Kellgren-Lawrence grade, functional assessment, conservative therapy record, BMI and (v2) optimization note.

If required documentation is missing, the request must be pended and the provider contacted for information. It must not be denied for lack of documentation until the information request has gone unanswered within the regulatory timeframe.

6. Version history

Version	Effective	Summary
v1	2024-01-01 to 2025-12-31	Initial version.
v2	2026-01-01	Hard BMI cut-off replaced by documented risk-optimization requirement.